

1. Introduction

1.1 Purpose

This Policy and the Policies and Procedures and related documentation supports {org.name} to apply the Risk Management and Safe Environment NDIS Practice Standards.

1.2 Policy Aims

{org.name} is committed to ensuring that:

- (a) risks to participants, workers and the provider are identified and managed; and
- (b) Each participant accesses support in a safe environment that is appropriate to their needs.

1.3 NDIS Quality Indicators

In this regard, {org.name} aims to demonstrate each of the following quality indicators through the application of this Policy and the relevant systems, procedures, workflows and other strategies referred to in this Policy and the Related Documentation:

Risk Management

- (a) Risks to the organisation, including risks to participants, financial and work health and safety risks, and risks associated with provision of support are identified, analysed, prioritised and treated.
- (b) A documented risk management system that effectively manages identified risks is in place, and is relevant and proportionate to the size and scale of the provider and the scope and complexity of support provided.
 - (1) The risk management system covers each of the following:
 - (2) incident management;
 - (3) complaints management and resolution;
 - (4) financial management;
 - (5) governance and operational management;
 - (6) human resource management;
 - (7) information management;
 - (8) work health and safety;
 - (9) emergency and disaster management.
- (c) Where relevant, the risk management system includes measures for the prevention and control of infection and outbreaks.
- (d) Supports and services are provided in a way that is consistent with the risk management system.
- (e) Appropriate insurance is in place, including professional indemnity, public liability and accident insurance.

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Approval Date: 16 October 2025	Next Scheduled Review: October 2027		

Safe Environment

- (a) Each participant can easily identify workers who provide support to them.
- (b) Work is undertaken with each participant, and others, in settings where supports are provided (including their home), to ensure a safe support delivery environment for them.
- (c) Where relevant, work is undertaken with other providers (including health care and allied health providers and providers of other services) to identify and manage risks to participants and to correctly interpret their needs and preferences.
- (d) For each participant requiring support with communication, clear arrangements are in place to assist workers who support them to understand their communication needs and the manner in which they express emerging health concerns.
- (e) To avoid delays in treatments for participants:
 - (1) protocols are in place for each participant about how to respond to medical emergencies for them; and
 - (2) Each worker providing support to them is trained to respond to such emergencies (including how to distinguish between urgent and non-urgent health situations).
- (f) Systems for escalation are established for each participant in urgent health situations.
- (g) Infection prevention and control standard precautions are implemented throughout all settings in which supports are provided to participants.
- (h) Routine environmental cleaning is conducted in settings in which supports are provided to participants (other than in their homes), particularly of frequently-touched surfaces.
- (i) Each worker is trained, and has refresher training, in infection prevention and control standard precautions including hand hygiene practices, respiratory hygiene and cough etiquette.
- (j) Each worker who provides support directly to participants is trained, and has refresher training, in the use of PPE.
- (k) PPE is available to each worker, and each participant, who requires it.

2. Definitions

In this Policy:

{org.name} means {org.legal_name} ABN {org.abn}.

Client means a client of {org.name} (including an NDIS participant).

Principal means {org.primary_contact.name} and {org.signatory.name}.

Infection requires three main elements — a source of the infectious agent, a mode of transmission and a susceptible host.

Infection control is preventing the transmission of infectious organisms and managing infections if they occur.

Infectious agents are biological agents that cause disease or illness to their hosts.

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Approval Date: 16 October 2025		Next Scheduled Review: October 2027	

Contact transmission usually involves transmission of an infectious agent by hand or via contact with blood or body substances. Contact may be direct or indirect.

Direct contact transmission occurs when infectious agents are transferred from one person to another, for example, a consumer's blood entering a healthcare worker's body through an unprotected cut in the skin.

Indirect contact transmission involves the transfer of an infectious agent through a contaminated intermediate object or person, for example, an employee touches an infected body site on one consumer and does not perform hand hygiene before touching another consumer.

Standard precautions are work practices which require everyone to assume that all blood and body substances are potential sources of infection, independent of perceived risk.

3. Principles

- (a) Effective infection control is central to providing high quality support for consumers and a safe working environment for {org.name} employees, Board members, students, and visitors. Staff and consumers are most likely sources of infectious agents and are also the most common susceptible hosts. Other people visiting the premises may be at risk of both infection and transmission.
- (b) The main modes for transmission of infectious agents are contact (including blood borne), droplet and airborne. Transmission of infection may also occur through sources such as contaminated food, water, medications, devices or equipment.
- (c) Infection control is integral to consumer support, not an additional set of practices.
- (d) Consumers' rights are respected at all times; they are involved in decision-making about their support, and they are sufficiently informed to be able to participate in reducing the risk of transmission of infectious agents.

4. Outcomes

- (a) Infections and infection transmission is prevented and managed as far as possible through the application of standard precaution practices.

5. Risk Management

- (a) Risks of infection are regularly assessed, identified and managed.
- (b) Employees are trained in infection control practice, including relevant application of precautions to minimise the risk of infection.
- (c) Mechanisms are in place for monitoring compliance with infection control procedures.

6. Policy Implementation

- (a) The organisation ensures effective implementation of infection control. All staff have access to policies and procedures relating to infection control. Tailored training is provided to persons with specific tasks where infection transmission is a risk.
- (b) Records of infection control activities are maintained, including infection control training undertaken, information provided to consumers and the use of personal protective equipment (PPE).
- (c) There are mechanisms for monitoring compliance with infection control.

7. Policy Detail

7.1 Infection Control Risk Management Plan

Approved By: {org.legal_name}	The Board of	Version	1
Approval Date: 16 October 2025		Next Scheduled Review: October 2027	

Once infection risks are identified, the organisation's risk management program includes:

- (a) Eliminating the risk factors
- (b) Modifying or changing procedures, protocols and work practices, whilst also monitoring consumer and employee compliance with infection control procedures
- (c) Providing information/education and training to consumers and employees.

7.2 Infection Risk Assessment

- (a) {org.name} identifies and assesses infection control risks by taking into consideration the likelihood of infection from a particular hazard, and the consequences if a person is infected. Factors such as frequency of exposure, levels of training and knowledge, existing controls, environmental factors and the experience of employees are considered.

The Managing and Reducing Known Risks Matrix prioritises identified risks for action.

{org.name} develops and prioritises actions for managing identified risks.

7.3 Education and Information

- (a) Education regarding infection prevention core principles is provided to all new staff and to existing staff annually.
- (b) Advice and information is provided to staff regarding new and emerging infectious disease threats and trends. Advice and education related to infection prevention is routinely provided to consumers.

7.4 Standard Precautions

- (a) Hand Washing and Hand Care
 - (1) Hand washing and hand care are considered the most important measures in infection control. Skin is a natural defence against infection. Cuts and abrasions on exposed skin should be covered with a water resistant dressing changed as often as necessary.
 - (2) Hands must be washed and dried before and after any direct consumer contact and/or the removal of gloves. Hands should be washed with a soap or cleaning agent covering all surfaces. Protective gloves must be worn when handling blood and body substances.
- (b) Protective Barriers
 - (1) Protective barriers (eye shields, gloves, gowns and masks) are to be used whenever there is a potential for exposure to blood and body substances.
 - (2) General purpose utility gloves should be worn for housekeeping tasks including: cleaning clinical instruments and handling chemical disinfectants.
 - (3) Utility gloves are to be discarded if they are peeled, torn or punctured or have other evidence of deterioration.
- (c) Needles and Sharps
 - (1) Special care must be taken to prevent injuries during procedures when cleaning sharp instruments, and use or disposal of sharps (needles). Sharps must not be passed from one worker to another unless specifically required for the proper conduct of the procedure.
 - (2) Needles must not be removed from disposable syringes for disposal nor resheathed before

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Approval Date: 16 October 2025		Next Scheduled Review: October 2027	

disposal. Where special circumstances require resheathing, it is preferable to use forceps or a protective guard.

- (3) Sharps containers should be placed as close as practical to the consumer care area, not easily accessible to visitors and out of the reach of children. Containers should be clearly labelled with the biohazard symbol and never overfilled.
- (d) Quarantining
 - (1) Staff, Board members, students, volunteers and consumers experiencing infectious conditions will be requested to refrain from {org.name} premises and activities during the infectious period of the condition.
- (e) Response to Possible Infection
 - (1) When potentially infected body fluids come into contact with an employee, Board member, student, volunteer or consumer, steps are taken to decrease the impact of such contact, including first aid and assessment at a medical service.

A supervisor must be notified of such incidents as soon as possible and an incident report form completed.
- (f) Notifiable Diseases
 - (1) {org.name} notifies relevant authorities in the event of an outbreak of any of the following: food borne illness in two or more related cases or gastroenteritis among people of any age in an institution.
- (g) COVID-19
 - (1) Refer to COVID-19 Pandemic Management Policy

8. General

8.1 Relevant Legislation, Regulations, Rules and Guidelines

Legislation, Rules, Guidelines and Policies apply to this Policy and Related Documentation as set out in the Legislation Register.

8.2 Inconsistency

If and to the extent that the terms of this Policy are or would be inconsistent with the requirements of any applicable law, this Policy is deemed to be amended but only to the extent required to comply with the applicable law.

8.3 Policy Details

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Version: 1

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